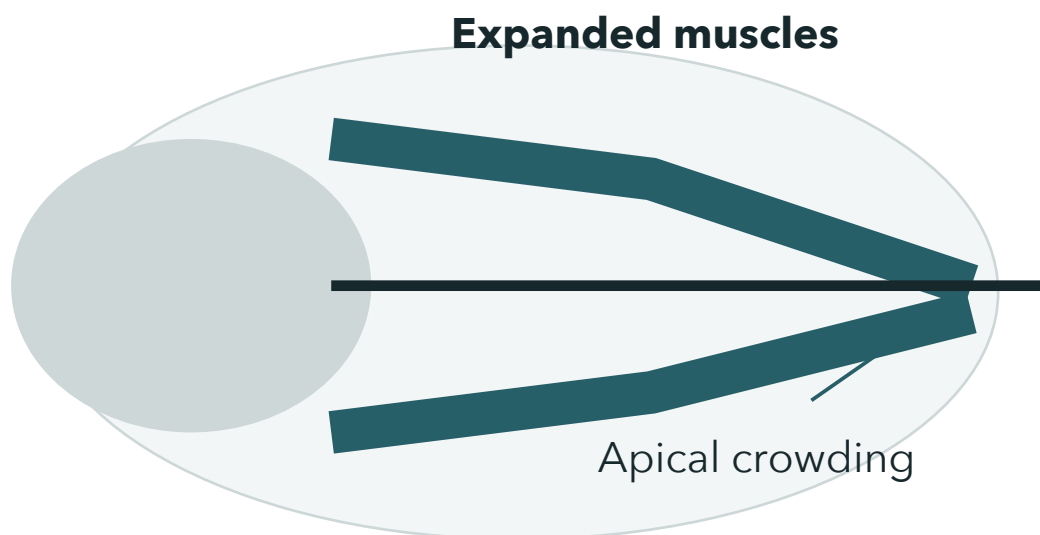


Thyroid eye disease activity treatment and risk

Part 2 | Activity and optic risk are separate axes

A modest external appearance can conceal apical crowding.



Original orbit section: expanded muscles can crowd the optic nerve at the apex.

Document visual function and optic-nerve threat separately from inflammatory activity and proptosis. Assess surface exposure, colour, fields and imaging when indicated. A low activity score does not provide complete reassurance about vision. State which process is being treated: inflammation, compression, exposure or a stable structural consequence; each needs a different intervention discussion.

Active moderate-to-severe disease

Match the patient to the recommended treatment context.

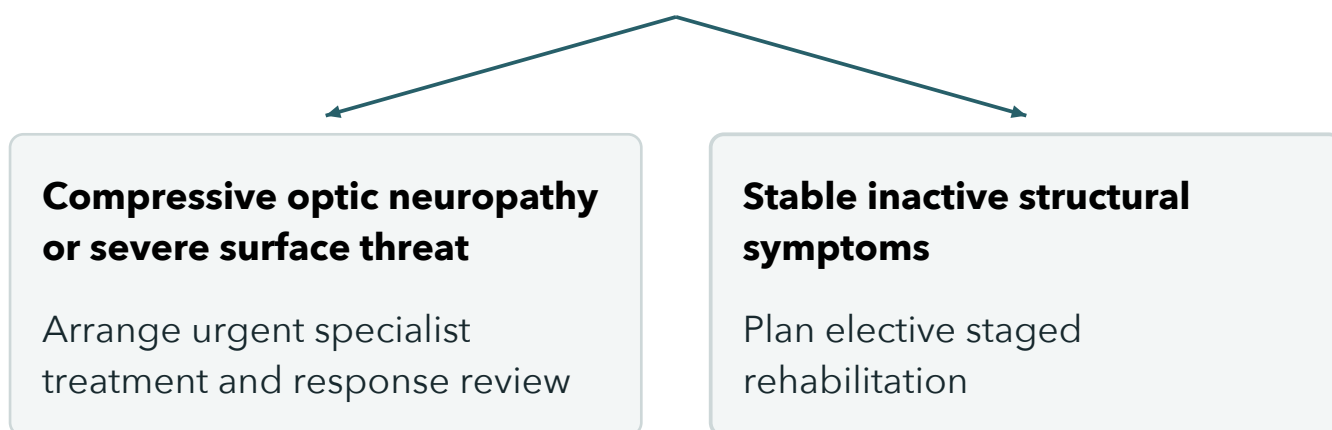
Finding	Implication
Usual EUGOGO first-line setting	IV glucocorticoid with mycophenolate
Greater severity or particular features	Alternative intensified IV steroid strategies may apply
Before systemic treatment	Review liver, cardiovascular and metabolic risk
Smoking and thyroid status	Address alongside ocular treatment

Avoid claiming a single regimen fits every active case. Specify severity, duration, dominant manifestations and contraindications. The guideline is a framework for multidisciplinary selection, with reassessment of benefit and toxicity. A treatment answer should include the intended target and monitoring, not just the name of an immunosuppressant.

Sight-threatening disease is a different pathway

Optic neuropathy and severe exposure override routine rehabilitation timing.

Is vision threatened now?



Explain why urgent treatment differs from correction of chronic proptosis or diplopia. A poor response to appropriate medical treatment can make decompression necessary in a compressive optic neuropathy. Protect the ocular surface simultaneously when exposure contributes. The aim is preservation of vision, with later appearance and binocular rehabilitation addressed after the emergency is controlled.

Teprotumumab: consent includes hearing

A targeted drug can have serious extraocular effects.

Finding	Implication
Hearing	Assess before, during and after treatment
Risk discussion	Hearing impairment may be severe or permanent
Metabolic context	Monitor relevant hyperglycaemia risk
Inflammatory bowel disease	Assess and monitor for exacerbation

Drug availability and recommended place in care differ by jurisdiction and can change. Use current local approval and product information. A baseline hearing history alone is not the same as the ongoing assessment required by the FDA label. Explain which new symptoms need prompt review and how toxicity could alter the course.

Rehabilitation: sequence prevents reworking the plan

Decompression can change motility; vertical surgery can change lids.

- Confirm stable inactive disease and agreed priorities
↓
- Perform indicated decompression first
↓
- Reassess alignment and plan strabismus correction
↓
- Correct residual lid position after preceding changes

The sequence applies when these procedures are needed, not as a requirement that every patient undergo all three. Prisms or occlusion may help while measurements stabilise. Counsel that primary-position single vision is a realistic objective even when normal movement cannot be restored. Consider residual reading-position symptoms and the effect of each stage on the next.